

Depression - Self-Care

CCG: C-0059 (CCG)

[Link to Codes](#)

MCG Health
Chronic Care
28th Edition

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Assessment

Initial Consideration

Q1. Patient: What would you like to talk about today? (Check all that apply.) **QM**

- Emergency or urgent problems (eg, social determinants of health, intimate partner violence, suicidal ideation)^[A] Social Determinants of Health Screening Assessment [CCG](#) Intimate Partner Violence [CCG](#) Suicide or Personal Harm Risk [CCG](#)
- Condition information
- Provider coordination (eg, next appointment, treatment clarification) Appointment Scheduling [CCG](#)
- Medications
- Complications
- Monitoring and management
- Other health problems (eg, comorbid conditions, new symptoms)
- Other, specify: _____
- Nothing
- Unknown

Q2. Patient: Do you have any changes since the last time we talked? (Check all that apply.) **QM**

- Adhering to treatment or medication problems Readiness to Change [CCG](#) Medication Adherence [CCG](#)
- Administering medication Home Infusion Management [CCG](#) Injectable Medications [CCG](#)
- Affording medicine or supplies Financial Status and Benefits [CCG](#)
- Aspiration Aspiration Risk Management [CCG](#)
- Behavioral health provider appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Blood pressure care Blood Pressure Monitoring [CCG](#)
- Blood sugar care Blood Glucose Monitoring [CCG](#) HbA1c Monitoring [CCG](#)
- Blood thinner care Anticoagulation Management [CCG](#)
- Bowel care Bowel Management [CCG](#)
- Bowel changes Bowel Symptoms [CCG](#)
- Caregiver Caregiver Resources Evaluation [CCG](#)
- Chemotherapy Chemotherapy [CCG](#)
- Cognitive impairment Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Communication or speech problems Speech and Communication Problems [CCG](#)

- Community resource Community Resources [CCG](#)
- Dehydration care Dehydration Monitoring and Management [CCG](#)
- Depression symptoms Depression Screening, Adult [CCG](#)
- Dialysis care Dialysis Management [CCG](#)
- Doctor or specialist outpatient appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Energy level changes Energy Level Changes [CCG](#)
- Enteral nutrition care Enteral Nutrition Management [CCG](#)
- Exercising and staying active Exercise [CCG](#)
- Fatigue or weakness Fatigue or Weakness [CCG](#)
- Fluid restriction Fluid Restriction [CCG](#)
- Functional status changes Activities of Daily Living [CCG](#) Instrumental Activities of Daily Living [CCG](#)
- Home infusion Home Infusion Management [CCG](#)
- Hospital or emergency room visit, specify: _____ Hospitalization Risk Assessment [CCG](#) Post-Hospitalization Follow-Up [CCG](#)
- Medication changes Medication Changes [CCG](#)
- Nebulizer care Nebulizer Management [CCG](#)
- Neurologic function changes Neurologic Function Changes [CCG](#)
- Ostomy care Ostomy Management [CCG](#)
- Oxygen care Oxygen Management [CCG](#)
- Pain, chronic Chronic Pain [CCG](#)
- Pain, new or general Pain Assessment [CCG](#)
- Parenteral nutrition care Parenteral Nutrition Management [CCG](#)
- Peak flow meter care Peak Flow Meter Use [CCG](#)
- Peripheral vascular (eg, lower leg redness or swelling) Peripheral Vascular Problems [CCG](#)
- Physical impairments Physical Impairments [CCG](#)
- Planning for long-term care needs Advance Care Planning [CCG](#)
- Procedure, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Pulmonary treatments Pulmonary Treatments [CCG](#)
- Rehabilitation therapy, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Respiratory changes Respiratory Symptoms [CCG](#)
- Safety Safety [CCG](#)
- Scheduling care Appointment Scheduling [CCG](#)
- Sleeping problems Sleep [CCG](#)
- Social support Social Support [CCG](#)
- Stasis ulcer care Stasis Ulcer Management [CCG](#)
- Testing (eg, laboratory, imaging), specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Tracheostomy care Tracheostomy Management [CCG](#)
- Transportation Transportation [CCG](#)
- Understanding healthcare needs Health Literacy [CCG](#)
- Urinary catheter care Urinary Catheter Management [CCG](#)
- Urinary changes Urinary Symptoms [CCG](#)
- Venous thromboembolism symptoms Venous Thromboembolism [CCG](#)
- Ventilator care Ventilator Management [CCG](#)
- Weight gain Nutritional Status, Adult [CCG](#) Nutritional Status, Pediatric [CCG](#) Nutritional Status, Infant [CCG](#)
- Weight loss Nutritional Status, Adult [CCG](#) Nutritional Status, Pediatric [CCG](#) Nutritional Status, Infant [CCG](#)
- Wound care Wound Management [CCG](#)

- Other, specify: _____
- No
- Unknown Hospitalization Risk Assessment [CCG](#)

Q3. Patient: Right now, do you have thoughts of suicide or harming yourself or someone else? **QM**

- Yes [B1] [P1]
- No
- Unknown [B1] [P1]

Q4. Patient: Have you had any of the following symptoms over the past 2 weeks? (Check all that apply.)

- Becoming frustrated or losing your temper easily [P2]
- Crying spells for no reason [P2]
- Eating too much or not wanting to eat at all [P2]
- Feeling guilty [P2]
- Feeling sad or unhappy a lot [P2]
- Feeling tired, sluggish, or physically drained [P2]
- Feeling worthless or hopeless [P2]
- Having trouble sleeping or sleeping too much [P2]
- Little or no interest in activities or hobbies that you enjoy, including sex [P2]
- Little or no interest in being with your friends and family [P2]
- Poor personal hygiene, not wanting to bathe, or not caring about what you wear [P2]
- Thoughts of death, suicide, or harming yourself or someone else [P2]
- Trouble concentrating or remembering things [P2]
- Trouble making decisions [P2]
- None
- Unknown [P2]

Q5. Patient: Have you ever tried to commit suicide? **QM**

- Yes [P2]
- No
- Unknown

Q6. Patient: What is your current diagnosis? (Check all that apply.)

- Bipolar disorder depression Bipolar Disorder - Self-Care [CCG](#)
- Depression with a seasonal pattern
- Dysthymic disorder
- Major depressive disorder
- Postpartum depression
- Other, specify: _____
- A depression-related diagnosis has not been established.
- Unknown [P3]

Condition Knowledge

Q1. Patient: What causes depression? (Check items understood.)

- Actual cause unknown
- Biochemical changes in the brain
- Family history (genetics)
- Life-changing event (eg, death of a loved one or chronic condition diagnosis)
- Stress
- Substance or alcohol misuse
- Other, specify: _____
- Unknown [P3]
- Nothing [P3]
- Not all appropriate information known [P3]

Q2. Case Manager: Rate the patient's understanding of the causes of depression. **QM**

- 90% or more
- 75% to 89% [P3]
- 50% to 74% [P3]
- Less than 50% [P3]
- Unknown [P3]

Q3. Patient: What are the symptoms of depression? (Check items understood.)

- Aches and pains for no reason
- Changes in eating habits
- Crying spells for no reason
- Feeling tired, sluggish, or physically drained
- Feeling worthless or hopeless
- Little or no interest in activities or hobbies
- Little or no interest in being with friends and family
- Poor personal hygiene, not wanting to bathe, or not caring about what you wear
- Thoughts of death, suicide, or harming self or someone else
- Trouble concentrating or remembering things
- Trouble making decisions
- Trouble sleeping or sleeping too much
- Other, specify: _____
- Unknown [P3]
- Nothing [P3]
- Not all appropriate information known [P3]

Q4. Case Manager: Rate the patient's understanding of the symptoms of depression. **QM**

- 90% or more
- 75% to 89% [P3]
- 50% to 74% [P3]
- Less than 50% [P3]
- Unknown [P3]

Q5. Patient: How can you care for depression to keep it from getting worse and causing problems? (Check items understood.) **QM**

- Adhere to self-care plan
- Ask for help when feelings are overwhelming
- Exercise
- Follow a daily care plan
- Follow an eating (diet) plan
- Get all follow-up laboratory tests when due
- Keep a diary about feelings, activities, eating times, and medicines taken
- Keep all doctor appointments
- Keep a vaccination schedule
- Know when to contact the doctor
- Manage situation or event triggers
- Manage stress
- Seek emergency care for signs of crisis plan not working
- Set realistic expectations for feeling better, managing symptoms, and recovery
- Stop using alcohol, tobacco, or drugs
- Take medicines as ordered
- Other, specify: _____
- Unknown [P3]
- Nothing [P3]
- Not all appropriate information known [P3]

Q6. Case Manager: Rate the patient's understanding of how to care for depression. **QM**

- 90% or more
- 75% to 89% [P3] [P4]
- 50% to 74% [P3] [P4]
- Less than 50% [P3] [P4]
- Unknown [P3] [P4]

Q7. Patient: What activities would you be able to do if you didn't have problems with your depression? (Check all that apply.) **QM**

- Activities of daily living (ADL) without assistance [P4]
- Cook [P4]
- Drive [P4]
- Exercise [P4]
- Go out with friends [P4]
- Go to events [P4]
- Make and keep friends [P4]

- Participate in activities or hobbies [P4]
- Play sports [P4]
- Play with children or grandchildren [P4]
- Shop [P4]
- Travel [P4]
- Work [P4]
- Other, specify: _____ [P4]
- Unknown [P4]

Provider Information

Q1. Patient: What doctor manages your depression? QM

- Psychiatrist or psychologist, name and phone number: _____ [Q2]
- Behavioral health therapist, name and phone number: _____ [Q3]
- Primary doctor, name and phone number: _____
- Other specialist, name and phone number: _____ [Q5]
- Don't have doctor [P3]
- Unknown [P3]

Q2. Patient: When was your last visit with the psychiatrist or psychologist?

- Less than 1 month ago
- 1 to 3 months ago [P4]
- 4 to 6 months ago [P4]
- More than 6 months ago [P4]
- Unknown [P4]
- Never [P4]
- Not applicable

Q3. Patient: When was your last visit with a behavioral health therapist?

- Less than 1 month ago
- 1 to 3 months ago [P4]
- 4 to 6 months ago [P4]
- More than 6 months ago [P4]
- Unknown [P4]
- Never [P4]
- Not applicable

Q4. Patient: When was your last visit with your primary doctor? QM

- Within the past 3 months
- 4 to 6 months ago
- More than 6 months ago [P3]
- Unknown [P3]
- Never [P3]

Q5. Patient: When was your last visit to the specialist doctor?

- Within the past 3 months
- 4 to 6 months ago [P3]
- More than 6 months ago [P3]
- Unknown [P3]
- Never [P4]

Q6. Patient: What other specialists have you seen within the last 12 months? (Check all that apply.) QM

- Behavioral health provider
- Cardiologist
- Dentist
- Dietitian
- Endocrinologist
- Gastroenterologist
- General surgeon
- Genetic counselor
- Geriatrician
- Gynecologist

- Hematologist
- Hepatologist
- HIV/AIDS
- Infectious disease
- Nephrologist
- Neurologist
- Neurosurgeon
- Obstetrician
- Oncologist
- Ophthalmologist
- Orthopedic surgeon
- Orthopedist
- Otolaryngologist
- Pain management
- Psychiatrist
- Plastic surgeon
- Podiatrist
- Pulmonologist
- Rheumatologist
- Transplant specialist
- Urologist
- Other, specify: _____
- None [P3]
- Unknown [P3]

Q7. Patient: How many emergency department visits or hospital admissions have you had in the past 12 months? **QM**

- 2 or more [P4] Hospitalization Risk Assessment [CCG](#)
- 1 [P4]
- None
- Other, specify: _____
- Unknown [P4]

Q8. Patient: Do you have a written self-care plan from your doctor on how to care for depression?[B] **QM**

- Yes [Q9]
- No [P3] [P4]
- Unknown [P3] [P4]

Q9. Patient: What are the components of your depression self-care plan? (Check all that apply.) **QM**

- Action plan for when symptoms start
- Counseling or therapy appointments
- Crisis management plan
- Exercise plan
- Following a routine for sleeping, waking up, personal care
- Knowing what medicines you take and why and how to take them
- Managing side effects of medicines
- Nutrition plan
- Recognizing the symptoms of gradual worsening or problems
- Regular follow-up appointments for wellness checks
- Support group attendance
- When to call the doctor and when to get emergency help
- Other, specify: _____ [P3]
- Unknown [P4]
- Not applicable
- Not all appropriate information known [P4]

Q10. Case Manager: Is the patient able to understand and adhere to the self-care plan? **QM**

- Yes, independently
- Yes, with occasional reminders or caregiver support
- Yes, with frequent reminders or caregiver support
- No [P5]
- Unknown [P5]

Medication Information

Q1. Patient: Has any medicine for depression been prescribed by the doctor? If so, is it taken as ordered? **QM**

- Yes, and taken as ordered [Q2]
- Yes, but not taken as ordered [Q2] [P6] Medication Adherence [CCG](#)
- No medicine prescribed [P6]
- Unknown [P6]

Q2. Patient: What medicines do you take for depression? (Check all that apply.) **QM**

- Atypical antidepressants (eg, bupropion, mirtazapine, trazodone) [Q3] [P3] [P7] [P8]
- Atypical antipsychotics (eg, aripiprazole, quetiapine) [Q3] [P3] [P7] [P9]
- Monoamine oxidase inhibitors (MAOIs) (eg, isocarboxazid, phenelzine, tranylcypromine) [Q3] [P7] [P10]
- Selective serotonin reuptake inhibitors (SSRIs) (eg, citalopram, fluoxetine, sertraline) [Q3] [P7] [P11]
- Serotonin-norepinephrine reuptake inhibitors (SNRIs) (eg, desvenlafaxine, duloxetine, venlafaxine) [Q3] [P7] [P11]
- Tricyclic antidepressants (TCAs) (eg, amitriptyline, clomipramine, trimipramine) [Q3] [P7] [P12]
- Other, specify: _____ [Q3] [P6] [P7]
- Not applicable [P6]
- Unknown [P7]

Q3. Patient: Are you having any side effects from the depression medicine? (Check all that apply.)

- Agitation [P7]
- Blurred vision [P7]
- Dark or hard stools (constipation) [P7]
- Dizziness [P7]
- Dry mouth [P7]
- Fast heartbeat [P7]
- Feeling sick to your stomach (nausea) or throwing up (vomiting) [P7]
- Headache [P7]
- Inability to sleep [P7]
- Inability to urinate [P7]
- Increase in suicidal thoughts [P7]
- Light sensitivity [P7]
- Sexual problems (abnormal ejaculation, impotence, or loss of orgasm) [P7]
- Sunburn [P7]
- Tiredness [P7]
- Weight loss or gain [P7]
- Other, specify: _____ [P7]
- None
- Unknown [P7]
- Not applicable

Q4. Case Manager: Rate the patient's understanding of the purpose and side effects of the medications for depression. **QM**

- 90% or more
- 75% to 89% [P3] [P7]
- 50% to 74% [P3] [P7]
- Less than 50% [P3] [P7]
- Unknown [P3] [P7]

Q5. Patient: Do you take any medicine that may make your depression worse? (Check all that apply.)

- Anabolic steroids [P5] [P6]
- Analgesics: opioids and NSAIDs [P5] [P6] Pain Assessment [CCG](#)
- Beta-blocker [P5] [P6]
- Calcium channel blocker [P5] [P6]
- Clonidine [P5] [P6]
- Corticosteroids [P5] [P6]
- Digoxin [P5] [P6]
- H2-receptor antagonist [P5] [P6]
- Interferon [P5] [P6]
- Other, specify: _____ [P5] [P6]
- None

- Unknown [P5] [P6]

Monitoring and Management

Q1. Patient: Is there someone you talk to about your problems?

- Yes, name and relationship: _____
- No [B2] [P13]
- Unknown [B2] [P13]

Q2. Patient: Do you have someone to count on when you need help?

- Yes, name and relationship: _____
- No [B2] [P13]
- Unknown [B2] [P13]

Q3. Patient: Is there someone who can help you feel better when you are stressed, sad, or upset?

- Yes, name and relationship: _____
- No [B2] [P13]
- Unknown [B2] [P13]

Q4. Patient: Is there someone who accepts you for who you are, the good and bad?

- Yes, name and relationship: _____
- No [B2] [P13]
- Unknown [B2] [P13]

Q5. Patient: Is there someone you can count on to care about you no matter what is happening?

- Yes, name and relationship: _____
- No [B2] [P13]
- Unknown [B2] [P13]

Q6. Patient: Does your social support help you as much as you need? **QM**

- Yes
- No [B2] [P13]
- Unknown [B2] [P13]

Q7. Patient: What problems with depression have you had in the past month? (Check all that apply.)

- Alcohol misuse [P4] [P5] Alcohol Use  CCG
- Changes in behavior at work (eg, not getting your work done on time, problems with coworkers, or late to work a lot) [P4] [P5]
- Changes in behavior in school (eg, being late to school a lot, changes in your grades in classes, or acting out) [P4] [P5]
- Drug use [P4] [P5] Substance Use  CCG
- Physical symptoms that do not respond to treatment (eg, headaches or gastrointestinal symptoms) [P4] [P5]
- Other, specify: _____ [P4] [P5]
- None
- Unknown [P4] [P5]

Q8. Patient: Have you had any changes to your nutritional health in the past 3 months? (Check all that apply.)

- Loss of appetite [P4] Nutritional Status, Adult  CCG
- Overeating [P4] Nutritional Status, Adult  CCG
- Weight gain [P4] Nutritional Status, Adult  CCG
- Weight loss [P4] Nutritional Status, Adult  CCG
- Other, specify: _____ [P4] Nutritional Status, Adult  CCG
- No
- Unknown [P4] Nutritional Status, Adult  CCG

Q9. Patient: Do you get regular exercise? **QM**

- Yes
- No [P4]
- Unknown [P4]

Q10. Patient: Are there any current sleeping problems? (Check all that apply.) **QM**

- Falling asleep [P14]
- Staying asleep [P14]

- Waking up too early [P14]
- Not feeling rested after sleeping [P14]
- Other, specify: _____ [P14]
- None
- Unknown [P14]

Q11. Patient: What do you do to help sleep? (Check all that apply.)

- Decrease caffeine
- Don't eat big meals before bedtime
- Get daily exercise
- Go to bed and get up at routine times of the day
- Keep journal of thoughts
- Sleep in bed, not on chair or sofa
- Sleep in dark, quiet room
- Sleep with head elevated, not lying flat
- Stopped drinking
- Stopped smoking
- Take a warm bath or shower before bedtime
- Take medicines, specify: _____ [Q12]
- Took distractions out of bedroom (eg, TV)
- Use equipment (eg, oxygen or CPAP)
- Other, specify: _____
- Nothing
- Unknown [P14]
- Not all appropriate sleep promoting activities being used [P14]

Q12. Patient: Do the medicines work to help sleep?

- Yes
- No [P14]
- Unknown
- Not applicable

Q13. Patient: Do you use tobacco, alcohol, or drugs? (Check all that apply.) **QM**

- Yes, tobacco use Tobacco Use  CCG
- Yes, alcohol use Alcohol Use  CCG
- Yes, drug use Substance Use  CCG
- No
- Unknown Tobacco Use  CCG Alcohol Use  CCG Substance Use  CCG

Q14. Patient: Have you received treatment for the problems related to depression? (Check all that apply.)

- Bright light therapy
- Electroconvulsive therapy
- Transcranial magnetic stimulation
- Other, specify: _____ [P4]
- No
- Unknown [P4]

Q15. Patient: Are you having any problems with your depression self-care needs? (Check all that apply.) **QM**

- Affording medicine or supplies [P4] [P5] Financial Status and Benefits  CCG
- Behavior indicating cognitive impairment [P4] [P5] Cognitive Impairment  CCG Behavioral Health and Cognition Evaluation with Caregiver  CCG
- Blood pressure care [P4] [P5] Blood Pressure Monitoring  CCG
- Blood sugar care [P4] [P5] Blood Glucose Monitoring  CCG
- Blood thinner care [P4] Anticoagulation Management  CCG
- Caregiver help needed [P4] [P5] Caregiver Resources Evaluation  CCG
- Community resource information needed [P4] [P5] Community Resources  CCG
- Daily care of child [P4] [P5] Caregiver Resources Evaluation  CCG
- Exercising and staying active [P4] [P5] Exercise  CCG

- Fatigue or weakness [P4] [P5] Fatigue or Weakness  CCG
- Safety in the home [P4] [P5] Safety  CCG
- Social support [P4] [P5] Social Support  CCG
- Taking medicine as prescribed [P4] [P5] Medication Adherence  CCG
- Transportation for care [P4] [P5] Transportation  CCG
- Trouble gaining weight [P4] [P5] Nutritional Status, Adult  CCG Nutritional Management to Gain Weight  CCG
- Trouble losing weight [P4] [P5] Nutritional Status, Adult  CCG Nutritional Management to Lose Weight  CCG
- Trouble making lifestyle changes [P4] [P5] Readiness to Change  CCG
- Trouble understanding how to do self-care [P4] [P5] Health Literacy  CCG
- Other, specify: _____ [P4] [P5]
- No
- Unknown [P4] [P5]

Q16. Case Manager: Rate the patient's understanding of the monitoring and management of depression. **QM**

- 90% or more
- 75% to 89% [P3] [P4]
- 50% to 74% [P3] [P4]
- Less than 50% [P3] [P4]
- Unknown [P3] [P4]

Other Health Considerations

Q1. Patient: Do you have other conditions that might make depression worse? (Check all that apply.) **QM**

- Alcohol or substance misuse [P4] [P5] Substance-Related Disorders - Self-Care  CCG Substance Use  CCG
- Anxiety [P4] [P5] Anxiety - Self-Care  CCG
- Arthritis [P4] [P5] Arthritis - Self-Care  CCG
- Bipolar disorder [P4] [P5] Bipolar Disorder - Self-Care  CCG
- Cancer [P4] [P5] Cancer - Self-Care  CCG
- Chronic obstructive pulmonary disease (COPD) [P4] [P5] Chronic Obstructive Pulmonary Disease - Self-Care  CCG
- Coronary artery disease [P4] [P5] Coronary Artery Disease - Self-Care  CCG
- Dementia [P4] [P5] Dementia - Self-Care  CCG
- Diabetes [P4] [P5] Diabetes, Adult - Self-Care  CCG
- Heart attack [P4] [P5]
- Heart failure [P4] [P5] Heart Failure - Self-Care  CCG
- Hepatitis C [P4] [P5] Liver Disease - Self-Care  CCG
- HIV/AIDS [P4] [P5] HIV/AIDS - Self-Care  CCG
- Obesity or overweight [P4] [P5] Obesity, Adult - Self-Care  CCG
- Parkinson disease [P4] [P5] Parkinson Disease - Self-Care  CCG
- Physical inactivity [P4] [P5] Exercise  CCG
- Posttraumatic stress disorder (PTSD) [P4] [P5] Anxiety - Self-Care  CCG
- Pregnancy [P4] [P5] High-Risk Pregnancy - Self-Care  CCG
- Seizures [P4] [P5] Seizure Disorders - Self-Care  CCG
- Stroke [P4] [P5] Stroke - Self-Care  CCG
- Systemic lupus erythematosus (SLE) [P4] [P5]
- Thyroid disease [P4] [P5]
- Other, specify: _____ [P4] [P5]
- None
- Unknown [P3]

Q2. Case Manager: Rate the patient's understanding of the potential complications of depression. **QM**

- 90% or more
- 75% to 89% [P3] [P4]
- 50% to 74% [P3] [P4]
- Less than 50% [P3] [P4]

- Unknown [P3] [P4]

Patient Understanding Evaluation

Q1. Case Manager: Does the patient understand all the causes of depression?

- Yes
- No [P3]
- Unknown [P3]

Q2. Case Manager: Does the patient understand all the symptoms of depression?

- Yes
- No [P3]
- Unknown [P3]

Q3. Case Manager: Does the patient understand how to care for depression? **QM**

- Yes
- No [P3] [P4]
- Unknown [P3] [P4]

Q4. Case Manager: Does the patient know when and where to get help for problems?

- Yes
- No [P3] [P5]
- Unknown [P3] [P5]

Q5. Case Manager: Does the patient have the ability to understand and follow the self-care plan? **QM**

- Yes
- No [P5]
- Unknown [P5]

Q6. Case Manager: Does the patient understand the purpose and side effects of their depression medication?

- Yes
- No [P7]
- Unknown [P7]

Q7. Case Manager: Is the patient having trouble coping with their depression? **QM**

- Yes [P5]
- No
- Unknown [P5]

Q8. Case Manager: Does the patient agree to education? **QM**

- Yes
- No, explain why: _____ [P5] Readiness to Change  CCG
- Unknown [P5]

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.
- HEDIS Measures include(1):
 - Tobacco use is assessed and addressed.
 - Readmission risk is assessed and addressed.
 - Alcohol and substance misuse is assessed and addressed.

Plan of Care

Barriers

- B1.** Personal harm risk, immediate
- B2.** Inadequate social support

Problems

P1. Immediate personal harm (ie, suicide or harm to others) risk present

- **Goal:** Immediate behavioral health evaluation and intervention to prevent patient harming self or others
 - **Interventions:**

- **ASSIST: If patient is actively threatening harm to self or others, contact the crisis hotline or emergency services while maintaining communication with the patient.**(2) Harming another person can be physical, emotional, or sexual. (3)
- **ASSIST:** National Suicide Prevention Lifeline: 1-800-273-8255 or call, text, or live chat 988
- **COORDINATE:** Follow-up contact after behavioral health evaluation to review self-care plan and identify need for further assistance(4)(5)

P2. Potential personal harm (ie, suicide or harm to others) risk present

- **Goal:** Personal harm (ie, suicide or harm to others) warning signs and action plan better understood
 - **Interventions:**
 - **ASSIST: If patient is actively threatening harm to self or others, contact the crisis hotline or emergency services while maintaining communication with the patient.**(2) Harming another person can be physical, emotional, or sexual. [C](3)
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of behavioral health status
 - **ASSIST:** Identifying patient supports and setting up support system communication(2)
 - **ASSIST:** Creation of an emergency plan so family and caregivers know steps to take if personal harm symptoms present(2)
 - **ASSIST:** Identifying risks of suicide or harm to self or others(6)(7)(8):
 - Self or family history of suicide or suicide attempts
 - Recent stressful event (eg, recent exposure to another's suicide) or serious adverse childhood event
 - History of being bullied
 - Social isolation or withdrawal
 - Severe functional impairments
 - Spousal loss
 - Discrimination experience (eg, lesbian, gay, bisexual, transgender)
 - Demographic factors (eg, male, older age, non-Hispanic White or American Indian/Alaska Native race)
 - Mental health disorders
 - Triggers include physical pain, loss of support or respect from family or peers, interpersonal conflicts, painful memories, loneliness, helplessness, lack of self-worth.
 - **EDUCATE:** Behavioral health crisis care(6)(9):
 - Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue(10)
 - Call police if crisis involves violence.(10)
 - Keep the crisis hotline telephone number near the phone.
 - Keep list of names, addresses, and emergency telephone numbers for medical help and support individuals (eg, neighbor, relative, close friend).
 - **EDUCATE:** Suicide warning signs(11)(12):
 - Depression (eg, feeling sad, hopeless, overwhelmed)(13)
 - Giving away personal possessions, settling accounts, or other actions relating to disposition of estate in case of death
 - Mood instability (eg, extreme or rapid changes)
 - Reckless activities
 - Sense of feeling trapped with no way out
 - Sleep pattern changes (eg, hypersomnia, insomnia)
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Suicidal or homicidal actions or plans(10)
 - Social changes (eg, isolation, withdrawal)
 - **EDUCATE:** Suicide prevention measures(14):
 - Take any hint of suicide seriously, and seek immediate professional help.
 - Encourage immediate intervention (eg, patient call crisis line, caregiver call provider).(12)
 - Keep list of names, addresses, and emergency telephone numbers for medical help and support individuals (eg, neighbor, relative, close friend).
 - Restrict access to methods of suicide.[D](12)
 - **EDUCATE:** Suicidal ideation signs(12):
 - Expressing suicidal thoughts or actions, or potential for harm to self or others
 - Disclosing of suicide plan or plan to harm another
 - Warning signs for suicide attempt (eg, writing suicide note, giving away valued possessions)
 - Role playing (eg, standing on chair with noose)
 - Recurrent thoughts of death (not just fear of dying)

- Available means to execute plan of harm to self or others
- **EDUCATE: When to get emergency help(12):**
 - Suicidal or homicidal actions or plans
 - Psychotic behavior (eg, threatening)
 - Crisis plan not working(15)
- **EDUCATE: When to call the provider(12)(16):**
 - Depression (eg, feeling sad, hopeless, overwhelmed)(12)(13)
 - Self-harm (eg, abuse, mutilation)
 - Hallucinations or delusions increase.
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
- **EDUCATE: National Suicide Prevention Lifeline: 1-800-273-8255 or call, text, or live chat 988**
- **COORDINATE: Behavioral health services referral, if indicated(2)(12)**
- **COORDINATE: Psychiatrist referral, if indicated(2)(12)(17)**
- **COORDINATE: Community services referral, as needed**
- **COORDINATE: Protective services or legal authorities, as needed**
- **COORDINATE: Social worker referral, as needed(18)**
- **COORDINATE: Care team meeting, if needed(19)(20)**
- **COORDINATE: Communication between care team members and care plan reconciliation(19)(21)(22)**
- **COORDINATE: Follow-up contact to evaluate safety, referral status, and need for further assistance(4)(5)**

P3. Depression information needed

- **Goal:** Depression diagnosis better understood(23)(24)[N](#)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE: Definition:** Depression is an alteration in an individual's mood that affects eating, sleeping, and feelings about self and life.(25)
 - It can be expressed by feelings of sadness, hopelessness, and a pessimistic outlook on life.
 - Depression may coexist with and complicate management of other illnesses.
 - Depression may be a sign of other psychiatric conditions (eg, anxiety, phobias, substance misuse).(26)
 - Subsequent episodes common; recurrent episodes increase in intensity, with shorter time frames between episodes.(27)
 - **EDUCATE: Depression types:**
 - Bipolar depression is a mood disorder with unusual shifts in mood, energy, activity levels, and ability to carry out day-to-day tasks.(12)(26)
 - Major depressive disorder (clinical depression) is a debilitating mood disorder with pervasive and persistent sadness, despair, and a diminished ability to think or make decisions.(12)(27)
 - Postpartum depression occurs within the first 4 weeks to 1 year after delivery; a type of major depression, signs are the same plus inability to care for child or desire to harm child.[E](28)
 - Dysthymia (persistent depressive disorder) is a chronic mildly depressed mood; less severe symptoms than major depression but still affects capacity to function.(27)
 - Seasonal depression (seasonal affective disorder (SAD)) is a combination of biological and mood disturbances affected by the changing seasons; depressed symptoms usually start in winter and subside in spring/summer.(12)
 - **EDUCATE: Causes(8):**
 - Exact cause is unknown.
 - Family history (genetics)
 - Psychosocial and environmental influences
 - Traumatic event history
 - **EDUCATE: Symptoms(27):**
 - Concentration difficulty
 - Thought process problems (eg, planning, reasoning, decision making)
 - Appetite changes
 - Emotional difficulties
 - Fatigue
 - Hygiene poor
 - Interest in activities or hobbies lost
 - Sleep problems
 - Social changes (eg, isolation, withdrawal)
 - **EDUCATE: Diagnosis:**
 - Based on medical history, risk factors, physical examination, and diagnostic tests

- Behavioral health examination and history
- Positive screening instrument
- **EDUCATE: Treatment(26):**
 - Psychotherapy (eg, cognitive behavioral therapy (CBT), individual or group therapy, family therapy)(29)
 - Self-care activities (eg, diet, activity, medication)
 - Medication management
 - Comorbid condition management
 - Symptom management
- **EDUCATE: Treatment goals(8):**
 - Complication and progression management
 - Functional improvement and maintenance (eg, ADL, IADL)(30)
 - Psychosocial functioning improved and maintained
 - Quality of life improvement
 - Recurrence prevention
 - Relapse reduction
 - Safety (for self and others)
 - Symptom frequency and severity reduced
- **EDUCATE: Resources:**
 - National Alliance on Mental Illness: www.nami.org
 - National Suicide Prevention Lifeline: www.988lifeline.org
 - Smokefree. National Institutes of Health: www.smokefree.gov
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated(26)
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Social worker referral, as needed(12)(18)
- **COORDINATE:** Care team meeting, if needed(19)(20)
- **COORDINATE:** Communication between care team members and care plan reconciliation(19)(21)(22)
- **COORDINATE:** Follow-up to evaluate depression understanding, referral status, and need for further assistance(4)(5)
- **SEND:** Your care plan - Depression  (Spanish)
- **SEND:** What you need to know - Depression  (Spanish)
- **SEND:** When to get help - Depression  (Spanish)
- **SEND:** Daily care for your condition - Depression  (Spanish)
- **SEND:** Be ready for your doctor visit - Depression  (Spanish)
- **SEND:** Be ready with questions for your doctor - Depression  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)

P4. Depression self-care needed

- **Goal:** Depression self-care plan developed, implemented, and maintained(23)(31)(32)
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help)
 - **ASSIST:** Communication between multiple providers(33)
 - **EDUCATE: Self-care plan information(25):**
 - Appointment scheduling and attendance
 - Adherence to plan necessary to achieve goals, and complications of nonadherence (eg, exacerbations, progression, comorbidities)(34)
 - Medication purpose, use, effects, and considerations (eg, timing before meals or exercise)(13)
 - Set small goals every day (eg, get out of bed at a certain time, finish one chore or project, or do something that brings pleasure like listen to music).(16)
 - Health checks, diary, and action plans (eg, write down thoughts, feelings, daily activities, eating times, and when medication taken)
 - Behavioral health treatment (eg, cognitive behavioral therapy (CBT), individual therapy, family therapy)(12)(29)
 - Symptom management
 - Relapse signs (eg, insomnia, decreased appetite, poor personal hygiene)(12)
 - Caregiver support plan, as needed

- Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue(35)
- Diet considerations (eg, limit caffeine and alcohol, avoid foods containing tyramine if taking an MAOI)(12)(36)
- Exercise program within provider's recommendations(29)(37)(38)N
- Sleep hygiene (eg, regular pattern, calm sleep environment)(2)
- Stress management (eg, relaxation and coping techniques)
- Substance use avoidance(2)
- Trigger avoidance (eg, high-stress situations, sleep deprivation)(12)
- Functional changes care (eg, adaptations and support for ADL and IADL)(30)
- Comorbid condition management(8)
- Wellness checkups (eg, vaccinations, screenings)(2)
- Laboratory monitoring, as needed
- Safety awareness and management (eg, remove suicide risks, de-escalation techniques)(39)(40)
- Support group participation(12)
- Medical alert bracelet/necklace, obtain and wear
- Tobacco cessation plan(41)N
- **EDUCATE: When to get emergency help(12):**
 - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(42)
 - Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(43)
 - Suicidal or homicidal actions or plans(26)(44)
 - Fatigue, extreme
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)(45)
 - Crisis plan not working
 - Delirium (eg, reduced awareness, agitation)(8)
 - Hallucinations or delusions, increased or new
 - Psychotic behavior (eg, threatening)
 - Seizures, new or complicated (eg, seizure longer than 5 minutes, multiple seizures with loss of consciousness, head injury during seizure)(46)
- **EDUCATE: When to call the provider for behavior changes(13):**
 - Depression (eg, feeling sad, hopeless, overwhelmed)(12)(13)
 - Fatigue, unrelenting
 - Insomnia
 - Mood changes (eg, depression, anger, agitation)
 - Self-care changes (eg, ADL or IADL problems)(30)
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Appointments missed (eg, support group, provider)
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)(29)
 - Life event, major (eg, job loss, death of someone close)
- **EDUCATE: When to call the provider(13):**
 - Constipation
 - Gastrointestinal distress (eg, nausea, vomiting, diarrhea, cramping)
 - Heart rate outside of provider's parameters
 - Medications newly ordered from other providers
 - Medication problems (eg, side effects, access, administration)
 - Pain changes (eg, new or worsening, unresolved with treatment)
 - Pregnant or planning to get pregnant(28)
 - Weight gain or loss without reason
- **COORDINATE:** Crisis services referral, as needed
- **COORDINATE:** Telehealth referral, if indicated[F](47)
- **COORDINATE:** Behavioral health services referral, if indicated(48)
- **COORDINATE:** Psychiatrist referral, if indicated(26)
- **COORDINATE:** Home care referral, if indicated(2)
- **COORDINATE:** Community services referral, as needed(49)
- **COORDINATE:** Social worker referral, as needed(12)(18)
- **COORDINATE:** Support group referral, as needed(50)
- **COORDINATE:** Vocational rehabilitation referral, as needed(51)
- **COORDINATE:** Tobacco use treatment, as needed
- **COORDINATE:** Care team meeting, if needed(19)(20)
- **COORDINATE:** Communication between care team members and care plan reconciliation(19)(21)(22)

- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(4)(5)
- **SEND:** Your care plan - Depression  (Spanish)
- **SEND:** What you need to know - Depression  (Spanish)
- **SEND:** When to get help - Depression  (Spanish)
- **SEND:** Daily care for your condition - Depression  (Spanish)
- **SEND:** Be ready for your doctor visit - Depression  (Spanish)
- **SEND:** Be ready with questions for your doctor - Depression  (Spanish)
- **SEND:** Caregiver tips  (Spanish)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** Help with sleeping or insomnia  (Spanish)
- **SEND:** How to stop smoking  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)
- **SEND:** When you are pregnant - Depression  (Spanish)
- **SEND:** Symptoms diary (Illustrated)  (Spanish)
- **Goal:** Depression therapy initiated and maintained (if applicable)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE:** Guided adjustment of depression self-care plan as indicated by provider. Treatment options(26):
 - Medication management
 - Behavioral health treatment (eg, cognitive behavioral therapy (CBT), individual therapy, family therapy)(29)(52)
 - Bright light therapy(27)
 - Electroconvulsive therapy (ECT)
 - Transcranial magnetic stimulation (TMS)(27)(29)
 - **EDUCATE:** Check with provider before using natural or herbal treatments.
 - **EDUCATE:** Bright light therapy information, if appropriate(27)(38):
 - Light therapy is used as a first-line treatment for seasonal affective disorder (SAD).
 - Ocular exposure to intense levels of artificial light (eg, 2500 lux for 2 hours)
 - Symptom improvement may be seen in 1 to 2 weeks.
 - Relapse may occur if treatment is discontinued.
 - Side effects include headache, eyestrain, nausea, irritability, photophobia, and insomnia (when used late in the day).(13)
 - **EDUCATE:** Electroconvulsive therapy (ECT), if appropriate(53):
 - Used primarily as treatment for severe major depression, catatonia, psychotic depression, or treatment-resistant depression
 - ECT is safe and may be more effective than pharmacotherapy.
 - Maintenance ECT is common.
 - Side effects include temporary memory loss and confusion.
 - Risks include mortality for patient with compromised cardiac status and permanent memory loss.
 - Relapse possible
 - **EDUCATE:** Transcranial magnetic stimulation (TMS), if appropriate(38)(54):
 - A brief and intense magnetic field within a coil resting on the scalp to painlessly penetrate human tissue and induce cortical neural activation or inhibition
 - It does not require anesthesia, and it has not been associated with adverse memory effects.
 - It is contraindicated with cochlear implants, deep brain stimulators, and vagus nerve stimulators.
 - Use caution with metallic hardware or implanted magnetic-sensitive medical devices (eg, implanted cardioverter-defibrillator, pacemaker, metal aneurysm clips or coils).(25)
 - **COORDINATE:** Behavioral health services referral, if indicated
 - **COORDINATE:** Psychiatrist referral, if indicated(26)
 - **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated(29)
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Care team meeting, if needed(19)(20)
 - **COORDINATE:** Follow-up contact to review understanding of depression treatment options and need for further assistance(4)(5)

P5. Depression potential complications and comorbidities monitoring needed

- **Goal:** Depression potential complication or comorbidity evaluated and addressed(23)(N)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of problems, comorbidities, and self-care plan[G]:
 - Recently hospitalized patient's evaluation within 1 week of discharge(N)
 - Recently hospitalized patient's evaluation with outpatient healthcare providers (eg, nurse, social worker) within 1 to 3 days of discharge(4)
 - Appointment every 3 to 6 months when patient is symptom free
 - Appointment once a month or more for patient with questionable adherence to treatment plan
 - **EDUCATE:** When and how to get help for worsening depression problems
 - **EDUCATE:** Treatment and monitoring for comorbid conditions (eg, coronary artery disease, high blood pressure, heart valve disease)(27)
 - **EDUCATE:** Follow-up appointments for assessment of clinical status
 - **EDUCATE:** Treating depression can help improve outcomes of coexisting chronic medical conditions.
 - **EDUCATE:** Treatment of coexisting chronic medical conditions may alleviate or eliminate depression.(8)
 - **EDUCATE:** Medications prescribed for medical or other behavioral health conditions can cause depression(8)(13):
 - Corticosteroids and hormones
 - Interferon
 - Chemotherapeutic agents
 - Levodopa
 - Antipsychotics
 - Sedative: barbiturates, benzodiazepines
 - Analgesics: NSAIDs, opioids
 - H2-receptor antagonists
 - Antihypertensives: beta-blockers, calcium channel blockers
 - **EDUCATE:** Medications for pre-existing conditions may need to be changed (type or dose).
 - **EDUCATE:** Certain medications prescribed for depression may worsen chronic medical conditions.(55)
 - **EDUCATE:** Effects of behavioral health condition and conjunctive medication usage during pregnancy and lactation(28)
 - **EDUCATE:** Recurrence information(56):
 - Recurrence risks include 2 or more depressive episodes, significant functional impairment, residual symptoms, comorbid chronic health problems.
 - Treatment maintenance to lower risk of relapse and improve outcomes when symptoms subside
 - Antidepressant treatment cessation (under clinician supervision) possible for low-risk patients in remission
 - **COORDINATE:** Pharmacy medication support, if indicated
 - **COORDINATE:** Specialist referral, if indicated
 - **COORDINATE:** Treatment program referral, if indicated(49)
 - **COORDINATE:** Preconception specialist referral, as needed
 - **COORDINATE:** Care team meeting, if needed(19)(20)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(19)(21)(22)
 - **COORDINATE:** Follow-up contact to review self-care plan and need for further assistance(4)(5)
 - **SEND:** Be ready for your doctor visit - Depression 🇪🇸 (Spanish)
 - **SEND:** Be ready with questions for your doctor - Depression 🇪🇸 (Spanish)
 - **SEND:** Narcotic medicine side effects 🇪🇸 (Spanish)
 - **SEND:** Pain medicines - Addiction and withdrawal 🇪🇸 (Spanish)
 - **SEND:** Be ready for your doctor visit - Substance-related disorders 🇪🇸 (Spanish)
 - **SEND:** Be ready with questions for your doctor - Substance-related disorders 🇪🇸 (Spanish)

P6. Depression medication evaluation needed

- **Goal:** Depression medication evaluation completed and medication regimen initiated, if indicated
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of condition status
 - **EDUCATE:** When and how to get help for worsening depression problems
 - **EDUCATE:** Medication management needs common for depression

- **COORDINATE:** Follow-up after provider contact for evaluation of medication changes(4)(5)

P7. Depression medication management needed

- **Goal:** Medication information understood(57)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
 - **ASSIST:** Providing reconciled medication list to all providers. See Medication Reconciliation Tool  SR for more information.
 - **EDUCATE: Purpose:** Importance of understanding what each medication is for, how it should be taken, and what to do if dose is missed
 - **EDUCATE:** Medication indications, side effects, and considerations frequently change. See a drug reference for specific information not listed here.
 - **EDUCATE:** Administration techniques:
 - Route (eg, oral, intramuscular (IM), intravenous (IV))
 - With or without foods or liquids (eg, milk)
 - Timing of administration
 - Appropriate measuring device (eg, liquid medications)
 - Proper storage
 - **EDUCATE: Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - **EDUCATE: Side effects:** Management, and when to contact provider
 - Common side effects include gastrointestinal changes, dry mouth, fatigue, insomnia, rash, dizziness, and headache.
 - **EDUCATE: Considerations:**
 - Discuss medication with provider if pregnant, planning a pregnancy, or breast-feeding.
 - Comorbidities may be contraindications to the use of a drug (eg, liver disease, diabetes).
 - Health checks, diary, and action plans (eg, blood pressure, weight, heart rate)
 - Safety measures (eg, sun protection, driving, or operating machinery)
 - Contact provider or pharmacist prior to taking any other over-the-counter or prescribed medication.
 - **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)
 - **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)
 - **COORDINATE:** Communication between care team members and medication list reconciliation
 - **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(4)(5)
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
 - **COORDINATE:** Therapeutic drug level monitoring
 - **SEND:** Managing your medicines (Illustrated)  (Spanish)

P8. Atypical antidepressant medication management needed

- **Goal:** Atypical antidepressant medication initiated and maintained(12)(58)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Atypical antidepressant medications are used to decrease the frequency and intensity of depressive episodes.
 - **EDUCATE: Side effects:** Dry mouth, weight changes, insomnia
 - Trazodone or mirtazapine may cause somnolence.
 - Trazodone may cause priapism.
 - **EDUCATE: Considerations:** Avoid bupropion in patients with seizure disorders.
 - **SEND:** Antidepressant medicines  (Spanish)

P9. Atypical antipsychotic medication management needed

- **Goal:** Atypical antipsychotic medication initiated and maintained
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Atypical antipsychotics may be used together with antidepressants to treat major depressive disorder.(27)(59)

- Second-generation antipsychotic medication
- **EDUCATE: Call provider immediately:** Neuroleptic malignant syndrome (NMS) (eg, extreme muscle rigidity, high fever, tachycardia, tachypnea, diaphoresis, stupor, fluctuations in blood pressure)(60)
- **EDUCATE: Side effects:** Difficulty urinating; jaundice; shuffling walk; fainting spells; seizures; pharyngitis; muscle spasms; excitement or restlessness; jerky movements of head, face, or neck; tachycardia; weight gain; blurred vision(61)
 - Increased risk of metabolic side effects, including obesity, diabetes, and dyslipidemia(62)
- **EDUCATE: Considerations(62):**
 - Use cautiously with history of seizures, liver disease, heart disease, blood disorders, or pregnancy.(55)
 - Monitor blood glucose, cholesterol, and triglyceride levels.
 - Talk to provider before discontinuing medication.
- **COORDINATE:** Electrocardiogram (ECG) monitoring, if indicated(62)
- **COORDINATE:** Laboratory testing

P10. Monoamine oxidase inhibitor (MAOI) medication management needed

- **Goal:** Monoamine oxidase inhibitor (MAOI) medication initiated and maintained(12)(58)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** MAOIs are antidepressant medications that decrease the frequency and intensity of depressive episodes.(8)
 - **EDUCATE: Call provider immediately:** Serotonin syndrome and hypertensive crisis signs:
 - Serotonin syndrome (eg, tachycardia, diarrhea, hyperthermia, tremor, clonus, agitation, delirium)
 - Hypertensive crisis (eg, blood pressure of 180/120 mm Hg or higher; severe headache; motor or sensory deficits; somnolence; seizure; unexplained chest, abdominal, or back pain)
 - **EDUCATE: Side effects:** Jaundice, hallucinations, fainting, hyperexcitability, confusion, fever, disorientation, decreased sexual function
 - **EDUCATE: Considerations:**
 - Do not take any new medications (including over-the-counter medications) unless cleared with prescribing provider.
 - Life-threatening drug interactions may occur.(1)
 - Avoid foods with amino acid tyramine, as hypertensive crisis can occur if tyramine consumed with MAOIs. Tyramine is found in foods such as avocados, bananas, yogurt, aged cheese, yeast, and aged meat.(55)
 - Prevent sudden drop in blood pressure and related falls by rising slowly from sitting or lying down.
 - **SEND:** Antidepressant medicines  (Spanish)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)

P11. Selective serotonin reuptake inhibitors (SSRIs) or serotonin-norepinephrine reuptake inhibitors (SNRIs) medication management needed

- **Goal:** Selective serotonin reuptake inhibitors (SSRIs) or serotonin-norepinephrine reuptake inhibitors (SNRIs) medication initiated and maintained
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** SSRIs and SNRIs are antidepressant medications.(8)
 - Prevent anxiety symptoms, reduce relapse risk, and have less safety risks compared to benzodiazepines(63)
 - Effects may take several weeks.(59)
 - **EDUCATE: Call provider immediately:** If taking SSRIs and MAOIs or TCAs and serotonin syndrome signs present(64)(65):
 - Serotonin syndrome (eg, tachycardia, diarrhea, hyperthermia, tremor, clonus, agitation, delirium)
 - **EDUCATE: Side effects:** Agitation, headache, postural hypotension, sexual dysfunction (eg, abnormal ejaculation, impotence, loss of orgasm)(66)
 - SNRIs have fewer sexual side effects than SSRIs.
 - **EDUCATE: Considerations(66)(67):**
 - Take dose early in the day to help avoid insomnia at night.
 - Use cautiously with cardiac conditions due to possible elevation in blood pressure with SNRIs.
 - **SEND:** Help with sleeping or insomnia  (Spanish)
 - **SEND:** Antidepressant medicines  (Spanish)

P12. Tricyclic antidepressants (TCAs) medication management needed

- **Goal:** Tricyclic antidepressants (TCAs) medication initiated and maintained(58)(67)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** TCAs are antidepressant medications that decrease the frequency and intensity of depressive episodes.

- TCAs decrease the frequency and intensity of panic attacks.
- **EDUCATE: Call provider immediately:** If taking TCAs and MAOIs or SSRIs and serotonin syndrome signs present(64)(65):
 - Serotonin syndrome (eg, tachycardia, diarrhea, hyperthermia, tremor, clonus, agitation, delirium)
- **EDUCATE: Side effects:** Blurred vision, urinary retention, orthostatic hypotension, reduction in seizure threshold, tachycardia, light sensitivity, weight gain
- **EDUCATE: Considerations:** Avoid in patients with angle-closure glaucoma or urinary retention.

P13. Social support needed

- **Goal:** Social support improved(68)(69)^N
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Patient in establishment of a helpful social network(70):
 - Identification of individuals in support network
 - Engaging in social support system (gathering information, helping patient develop skills, encouraging shifts in perspective, and allowing patient to vent about issues)
 - **EDUCATE:** Ways to access social support
 - **EDUCATE:** Role of social support in quality of life (eg, improved self-image, coping)(69)
 - **EDUCATE:** Patient's role in social support relationships (eg, be aware of how actions impact the feelings of family and friends)(71)
 - **COORDINATE:** Behavioral health services referral, if indicated
 - **COORDINATE:** Community services referral, as needed(72)
 - **COORDINATE:** Social worker referral, as needed(18)
 - **COORDINATE:** Support group referral, as needed(73)(74)
 - **COORDINATE:** Support network contact (eg, church, social groups), as needed(70)
 - **COORDINATE:** Follow-up to evaluate social support, referral status, and need for further assistance(4)(5)

P14. Sleeping problems present

- **Goal:** Sleep improved^N
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Scheduling provider appointment for medical evaluation of sleep changes and creation of self-care plan to include(75)(76):
 - Identifying techniques to improve sleep quality(77)
 - Medication changes that can help patient sleep better (eg, changing the time stimulants are taken to early in the day or not taking diuretic before bedtime)
 - Review of herbal preparations that may promote sleep(78)
 - Creation of sleep diary(79)
 - **EDUCATE:** Sleep hygiene techniques(75)(77)(80):
 - Schedule sleep (go to bed and get up at same time every day).
 - Spend no more than 8 hours per day in bed. Amount of average sleep time for adult is 7 to 8 hours.
 - Limit napping to once a day for 20 to 30 minutes in early afternoon.
 - Sleep in a dark, quiet bedroom.
 - Use bedroom only for sleeping, not to watch TV, work, use electronics, or do other activities.
 - Avoid pets in the bed.
 - Get safe amount of sunlight every day and avoid bright lights before bedtime.
 - Cut down on drinking fluids in the afternoon if nighttime urination is a problem.
 - Avoid substances that may influence sleep (eg, caffeine, stimulants, alcohol).
 - Stop smoking; nicotine is a stimulant.
 - Get regular exercise, but avoid vigorous exercise 2 hours before bedtime.(81)
 - Write down urgent thoughts if mind-racing is a problem.
 - Do not eat big meals before bedtime; gastric process can cause sleep problems.
 - **EDUCATE:** Herbal preparations to help sleep (eg, melatonin, tryptophan, valerian), as approved by provider(82)^N
 - **EDUCATE:** Behavioral health treatment options and benefits (eg, cognitive behavioral therapy for insomnia)(79)(83)^N
 - **EDUCATE:** Hypnotic medications use and misuse, if prescribed for insomnia(76)
 - **COORDINATE:** Sleep specialist referral, if indicated(82)
 - **COORDINATE:** Behavioral health services referral, if indicated(79)

- **COORDINATE:** Communication between care team members and care plan reconciliation(19)(21)(22)
- **COORDINATE:** Follow-up contact to evaluate sleep, status of referrals, and need for further assistance(4)(5)
- **SEND:** Help with sleeping or insomnia  (Spanish)

Evidence Summary

Quality standards: The Case Management Society of America's Standards of Practice for Case Management principles include facilitation of informed decision making, counseling, and health education with patient self-care that is responsive to the patient's preferences (eg, culture, values, needs).(84)

Quality standards: Standards indicate that program content should include an individualized self-care plan that includes patient preferences, prioritized goals, interventions with information on whom to contact in urgent situations, and scheduled follow-up. Programs should facilitate communication and collaboration with practitioners in a timely manner.(84)(85)(86)(87)

National guidelines: The 2018 Physical Activity Guidelines Advisory Committee advises that adults with chronic conditions or disabilities, who are able, should follow the key activity guidelines for adults. The guidelines recommend that adults should do at least 150 to 300 minutes a week of moderate-intensity or 75 to 150 minutes a week of vigorous-intensity aerobic physical activity, or an equivalent combination of moderate-intensity and vigorous-intensity aerobic activity. They should also do muscle-strengthening activities on 2 or more days a week. Older adults should do multicomponent physical activity that includes balance training as well as aerobic and muscle-strengthening activities.(37)

Evidence: A randomized controlled study based on 526 patients found that smoking cessation can be achieved among people with severe mental illness without negatively impacting mental health (eg, depression, anxiety).(88)

Quality standards: Standards indicate that programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(84)(85)(86)(87)

Transition consideration: Several studies demonstrated a reduction in 30-day hospital readmissions when the patient follow-up was within 7 days of hospital discharge.(89)(90)(91) Earlier follow-up (eg, within 48 hours after hospital discharge) may also help reduce hospital readmissions for high-risk or vulnerable patients.(4) Follow-up appointment attendance rates were higher when appointments were scheduled before hospital discharge.(92)(93)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(94)

Quality standards: Standards indicate that program content should address patient supports and psychosocial issues that may impede self-management. Programs should have a process for following up with patients on the status of referrals.(84)

Quality standards: Standards indicate that program content should include a condition treatment plan, self-management and self-monitoring, and health behaviors (eg, sleep patterns). Programs should have a process for following up with patients on the status of referrals.(84)

Specialty society: In a systematic review of the pharmacologic treatment of chronic insomnia in adults, the American Academy of Sleep Medicine does not recommend diphenhydramine, melatonin, tryptophan, or valerian for treating sleep onset or maintenance.(78)

Specialty society: The American College of Physicians recommends cognitive behavioral therapy for insomnia as the first-line treatment for patients with chronic insomnia (trouble falling or staying asleep for 3 or more nights a week for at least 3 months, with daytime consequences). Cognitive behavioral therapy for insomnia generally includes cognitive therapy with sleep restriction, relaxation training, and stimulus control for 5 to 8 sessions.(95)

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Footnotes

[A] Assess and address the emergency or urgent problems before continuing the engagement. [A in Context Link 1]

[B] A self-care plan tells you how to take care of your condition and know when and where to get help for problems. [B in Context Link 1]

[C] Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Inpatient treatment may be appropriate for people with depression who are at significant risk of suicide, self-harm, or self-neglect.(2) Other patients who will reliably participate in monitoring and have supports and providers available may be safely treated with frequent monitoring in outpatient care.(6)(9) [C in Context Link 1]

[D] Restricting access to suicide methods includes removing firearms or other weapons from the home, keeping only immediate medication dosages available, removing materials that could be used for hanging or suffocation, and access restrictions for jumping (eg, closed-caption video monitoring, physical barrier to bridges).(11)(12)(39)(96) [D in Context Link 1]

[E] Two other depressive disorders specific to the postpartum period are mild postpartum blues and severe puerperal psychosis.(28) [E in Context Link 1]

[F] Telehealth may improve access to and coordination of mental health care. Patients with behavioral health diagnoses may use telehealth or telepsychiatry to access mental health support or care (eg, to get assistance with problem-solving techniques or ask about presenting symptoms or medication use).(97) [F in Context Link 1]

[G] The timing of provider follow-up depends on the patient's disease severity, self-care complexity, patient/caregiver willingness and ability to provide patient care, and available social supports.(98) [G in Context Link 1]

[H] Neuroleptic malignant syndrome is a rare, rapidly progressive, potentially fatal complication more common with typical antipsychotics. Onset can occur hours or years after initiation of drug therapy.(60) [H in Context Link 1]

[I] Medications to avoid include sympathomimetics (eg, amphetamines, combination cold tablets, nasal decongestants, and appetite suppressants), illicit drugs, narcotic medications (eg, meperidine, tramadol, and methadone), selective serotonin reuptake inhibitors (SSRIs), and tricyclic antidepressants (TCAs).(66) [I in Context Link 1]

Codes

ICD-10 Diagnosis: F06.30, F06.31, F06.32, F06.34, F10.14, F10.24, F10.94, F11.14, F11.24, F11.94, F13.14, F13.24, F13.94, F14.14, F14.24, F14.94, F15.14, F15.24, F15.94, F16.14, F16.24, F16.94, F18.14, F18.24, F18.94, F19.14, F19.24, F19.94, F32.0, F32.1, F32.2, F32.3, F32.4, F32.81, F32.89, F32.9, F32.A, F33.0, F33.1, F33.2, F33.3, F33.40, F33.41, F33.9, F34.0, F34.1, F34.89, F43.21, F43.23, F53.0, F53.1, O90.6, R45.88, Z13.31, Z13.32, Z91.51, Z91.52 [Hide]

DSM-5: F06.31, F06.32, F06.34, F10.14, F10.24, F10.94, F11.14, F11.24, F11.94, F13.14, F13.24, F13.94, F14.14, F14.24, F14.94, F15.14, F15.24, F15.94, F16.14, F16.24, F16.94, F18.14, F18.24, F18.94, F19.14, F19.24, F19.94, F32.0, F32.1, F32.2, F32.3, F32.4, F32.81, F32.89, F32.9, F32.A, F33.0, F33.1, F33.2, F33.3, F33.41, F33.9, F34.0, F34.1, F43.21, F43.23, R45.88 [Hide]

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